مستشفى الحديثة العام
Al Hadeethah General Hospital
Empowered by Aljouf Health Cluster

EARLY WARNING SIGNS AND SHOCK EMERGENCIES

Name:	**Job Number:**	**Rating:**
Unit:	**Job Title:**	
Contract Date:		
Evaluation Key: **M- Met NM- Not Met NA- Not Applicable**	**Method of Evaluation:** **Knowledge:** Exam(Written/Oral) **Skills:** Demonstration/Discussion **Attitude:** Observation	

Rating Scale: Met: 90% - 100% Not Met: 89% & below and remedial once NA-(Not applicable) – entries to be deducted from the total score

	COMPETENCIES	EVALUATOR ASSESSMENT M (1)	NM (0)	NA
I.	**KNOWLEDGE**			
1.	Explain the rationale for EARLY WARNING SIGNS i.e. to provide an integrated approach to the recognition and response to 'at risk' patients, preventing avoidable deterioration.	☐	☐	☐
2.	Explore how being aware of EARLY WARNING SIGNS will improve patient outcomes and why this is important in terms of the quality of life.	☐	☐	☐
3.	Explain why real time vital signs are important when a patient is recognized as 'at risk' e.g. initial baseline and on-going trends.	☐	☐	☐
4.	Discuss the EARLY WARNING SIGNS Tool developed for area of practice and explain how the EWS Scale works.	☐	☐	☐
5.	List the NURSING INTERVENTIONS AUTHORIZED once Early Warning Signs are identified and why these are important to patient outcomes NOTE: Patients hemodynamic status and assess airway, breathing and circulation by repeating VITAL SIGNS at least every 15 minutes.	☐	☐	☐
6.	Explain why it is necessary to ensure that the following items are ready / accessible: • Crash cart • Suction equipment • Oxygen delivery • IV set up and fluids • Dynamap with oxygen saturation probe	☐	☐	☐
7.	Define shock i.e. when cells become hypoxic as a result of decreased perfusion.	☐	☐	☐
8.	Discuss the categories and causes of shock: Hypovolemic – massive external bleeding, hemothorax, fractures, gastrointestinal bleeding, diabetes mellitus insipidus burns, excessive diuretic use, ascites, massive vomiting or diarrhoea Cardiogenic – MI, cardiomyopathy, cardiac contusion, dysrhythmias, heart valve disease Distributive – sepsis, anaphylaxis, spinal cord injury, overdose, anoxia Obstructive – tension pneumothorax, pericardial tamponade, pulmonary embolus, intra-cardiac clot, vena cava clot, aortic aneurysm, aortic stenosis, valvular disease, gravid uterus	☐	☐	☐
9.	Explore the cellular effects of shock e.g. < ATP (adenosine triphosphate), excess lactic acid production, mitochondrial death, cellular oedema, deterioration of the sodium-potassium pump, hyperkalaemia.	☐	☐	☐
10.	Discuss the progression of shock: • Compensatory – body responses initiated to increase cardiac output, tissue (cellular perfusion) and vital organ perfusion • Progressive – body responses become inadequate to maintain perfusion and multisystem failure ensues • Irreversible – pervasive cellular destruction; death is imminent	☐	☐	☐

	COMPETENCIES	EVALUATOR ASSESSMENT		
		M (1)	NM (0)	NA
11.	Explain the rationale for pertinent historical date: 1. Chief Complaint 2. Pain, pressure (PQRST) • Provocation • Quality • Radiation/region • Severity (scale 0-5 or 0-10) • Time of onset 3. Level of consciousness 4. Dizziness, syncope 5. Weakness, fatigue 6. Difficulty breathing 7. Edema – location and type 8. Vomiting, hematemesis 9. Diarrhea, melena 10. Vaginal bleeding 11. Fever, chills 12. Rash, urticarial 13. Polyuria, thirst 14. Injury, location, mechanism, force, protective device 15. Bleeding, site, estimated blood loss 16. Medical-surgical history 17. Allergies			
12.	Discuss the acid base deviations that occur: Respiratory Acidosis – Hypoventilation results in CO_2 retention, carbonic acid excess and <pH Metabolic Acidosis – Excess fixed acids results in <pH Respiratory Alkalosis – Hyperventilation results in >CO_2 exhaled, reducing carbonic acid with >pH Metabolic Alkalosis – >pH due to excess base or acid deficits			
13.	Explore the adverse effects of fluid and/or blood replacement: 1. Hypothermia – transfusion of banked blood without warming can make the patient hypothermic. Hypothermia shifts the oxyhemoglobin dissociation curve to the left i.e. oxygen is not available for the cells. 2. Hyperkalemia – lysis of blood cells releases potassium from the intracellular space so the patient becomes hyperkalemic. Monitor the patient's serum potassium and observe for cardiac rhythm problems. 3. Hypocalcaemia – banked blood contains citrate which binds with free calcium. Calcium chloride is given after 10U of blood. 4. Acidosis – the pH of banked blood is 7.1 and with large amounts of banked blood, acidosis can occur. Monitor ABGs and watch for dysrhythmias. 5. Alkalosis – large amounts of banked blood, alkalosis can develop because the citrate is converted by the liver into bicarbonate. 6. Clotting problems – clotting factors are lost in most banked blood. Coagulation times may be prolonged and clotting problems occur with massive blood transfusions. Usually 1U of fresh-frozen plasma is given after 10U of blood. 7. Intravascular debris – banked blood contains debris as a result of processing and therefore blood is always given through a filter.			
14.	Explain how to document patient assessment in the patient forms and notes according to area of practice.			
II.	**SKILLS**			
1.	Determine why real time vital signs are important when a patient is recognized as 'at risk' e.g. initial baseline and on-going trends.			
2.	Demonstrate the EARLY WARNING SIGNS Tool developed for area of practice and show how the EWS Scale works.			
3.	Carry out the NURSING INTERVENTIONS AUTHORIZED once EWS are identified and demonstrate why these are important to patient outcomes NOTE: Patients hemodynamic status and assess airway, breathing and circulation by repeating VITAL SIGNS at least every 15 minutes			

	COMPETENCIES	EVALUATOR ASSESSMENT M (1)	NM (0)	NA
4.	Demonstrate that the following items are ready / accessible: • Crash cart • Suction equipment • Oxygen delivery • IV set up and fluids • Dynamap with oxygen saturation probe	☐	☐	☐
5.	Draw the cellular effects of shock e.g. < ATP (adenosine triphosphate), excess lactic acid production, mitochondrial death, cellular oedema, deterioration of the sodium-potassium pump, hyperkalaemia	☐	☐	☐
6.	Demonstrate the patient assessment for Stage 1 of shock: Stage I – Compensatory Increased heart rate (101-150 bpm) Skin: pale, moist, clammy Pupils: dilated (6-8 mm, react to light appropriately) Blood pressure: usually normal at this stage Heart rhythm: sinus tachycardia Respirations: increased (>24 bpm) Peripheral pulse: weak Altered level of consciousness: confused, anxious, restless, lethargic Glasgow Coma Scale Poor capillary refill (>2 seconds) May have petechiae or purpura Depending on the cause may be at risk for infection *This is the stage with the best chance of reversibility	☐	☐	☐
7.	Demonstrate the patient assessment for Stage 2 of shock: Stage II – Progressive Increased heart rate Skin: cool, moist, clammy Pupils: dilated, sluggish response Blood pressure: starting to drop Heart rhythm: sinus tachycardia Respirations: rapid and deep Altered level of consciousness worsens Decreased urinary output Peripheral edema Dysrhythmias	☐	☐	☐
8.	Demonstrate the patient assessment for Stage 3 of shock: Stage III - Irreversible Decreased heart rate Skin: cold Pupils: dilated, very sluggish Severe hypotension Severe bradycardia Respiratory failure Possible coma Anuria Peripheral edema Death	☐	☐	☐
9.	Demonstrate appropriate action when readings and vital signs and patient respiratory assessment findings are abnormal including the documentation of findings and action taken.	☐	☐	☐
10.	Complete the documentation for clinical management and patient assessment in the patient forms and notes according to area of practice.	☐	☐	☐
III.	**ATTITUDE**			
1.	Accept constrictive criticism as learning opportunity.	☐	☐	☐

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	COMPETENCIES	EVALUATOR ASSESSMENT M (1)	NM (0)	NA
2.	To be self-aware of limitation in knowledge and skills.			
3.	To be approachable and open to answer patient and family's questions and ability to educate both patient and family how to keep any IV lines etc. secure and in place.			
4.	Attentive to overall patient experience e.g. pain, anxiety and have ability to provide comfort and emotional support as needed.			
5.	Ability to organize and facilitate a Reflection session in and on practice after a MANAGEMENT AND CARE OF SHOCK learning opportunity occurs and activate findings to improve future practice and response.			
	Raw Score			

Formula: $\frac{\text{Raw Score}}{\text{Total Score}} \times 100\% =$ ______ **% Rating**

NEEDS REMEDIAL: ☐ **YES** ☐ **NO**
REMEDIAL DATE:

Evaluators Comments/Recommendations:

Evaluated By:

Evaluator's Name/Signature/Job Number

Staff Nurse Comments:

Conformed By:

Staff Name/Signature **Date**

تجمع الجوف الصحي
Aljouf Health Cluster